DOCUMENT 1 OF 2 · CONTENT STRATEGY

The Dr. Azka Content Bible

10 content pillars · 50 subtopics · 150 ready-to-script angles for clinic owners: dermatologists, cosmetic dentists and aesthetic physicians in the UAE & USA. With competitor gaps and the Value-Per-Second scripting system.

Prepared for Dr. Azka · Doctor-led content & AI growth agency, Dubai
Built from: your positioning, ICP, business model, funnel and offer documents, your 210-reel keyword research and your 76-reel competitor analysis.
Prepared 31 October 2026

1. How to use this document

This is the content plan for **your own personal brand**, the account that makes clinic owners think: *"She's a doctor, she understands my world, she knows how content brings patients, and she builds the systems that stop them leaking."* Every pillar below exists to move a dermatologist, cosmetic dentist or aesthetic clinic owner one step along your funnel:

> **Your signature line (repeat it everywhere)**
> "Content gets you the opportunity. Your systems decide what happens to it."

- **Pillar → Subtopic → Angle.** Each angle is written as a hook-ready idea. Tags show the suggested **format** and the **funnel stage**: TOFU discovery, MOFU trust, BOFU conversion.
- **Every pillar page** tells you what your competitors are doing, the **gap** they leave, how you **win** it, and one **value-per-second** rule for that pillar.
- **150 angles = about 3–4 months of content** at 1–2 posts/day. Each angle can also become a carousel, a LinkedIn post and an email, so you'll get far more than 150 pieces.
- **Swap the specialty.** Most angles work for all three specialties. Change 'dermatologist' to 'cosmetic dentist' or 'aesthetic doctor' to make three versions.

Your content mix (it keeps the 80/20 rule from your brand document)

#	Pillar	Share	Stage
1	What Dermatologists, Cosmetic Dentists & Aesthetic Doctors Should Actually Post	15%	TOFU → MOFU
2	Hooks, Scripting & Value-Per-Second (The Craft)	10%	TOFU
3	Views → Patients (Content as a Patient-Acquisition Channel)	15%	MOFU
4	Patient Psychology: How Patients Choose a Dermatologist or Cosmetic Dentist	10%	TOFU → MOFU
5	Evidence-Based & Compliant Medical Marketing (UAE + USA)	5%	MOFU
6	The Patient Leak: Inquiry → Booking (Speed, DMs, Front Desk)	10%	MOFU → BOFU
7	AI Systems for Clinics (Receptionist, Booking, No-Shows)	15%	MOFU → BOFU
8	Retention, Reactivation & Reviews (The Money Already in the Database)	5%	MOFU → BOFU
9	Clinic Growth Economics, Teardowns & Proof	5%	MOFU → BOFU
10	Dr. Azka: A Doctor Building for Doctors (Human Layer)	10%	TOFU → MOFU

Mapping to your original 5 pillars: Doctor Content Strategy = Pillars 1, 2 & 4 · Content → Patient Acquisition = 3 & 6 · AI & Automation = 7 & 8 · Clinic Growth = 5 & 9 · Human Layer = 10 (and the story moments inside every pillar). Pillar 5 (Compliance) is new: it's the biggest gap your research revealed.

2. What your competitors do, and the gaps you fill

This is a summary of the 76 competitor reels and 210 keyword reels you analysed. One caveat: I worked from your written analysis (hooks, formats, CTAs, white spaces). I didn't watch the videos myself, and several reels in your sheet were marked inaccessible or age-restricted.

Competitor	What they do	What they DON'T do	How you beat them
Amra Farooq	Doctor personal branding; 'relevance over reach'; funnel and framework talks; heavy 'DM brand' CTAs	Hides the framework behind DMs; no clinic proof; no conversion numbers; compliance checklist withheld	Show the full framework in the video. Show patient-inquiry numbers. Give the jurisdiction-specific compliance content for free.
Sam R Patel	Front-desk calls, greetings, clinic systems rankings; UK; event promos	Rarely names fixes, scripts or metrics; almost no CTAs; no AI layer	Front-desk content + an AI demo that fixes the leak + a script they can steal.
Dr. Nickc	Dental marketing; GBP, testimonials, KPIs, AI; lots of lead magnets ('LIMITLESS', 'GUIDE')	Teaser hooks ('if you know these...') without naming the KPIs; no live AI build	Name the KPIs and show the formulas. Show the AI build with guardrails.
Maha Chalawi	Esthetician / med-spa business; luxury service; loyalty; paid masterclass	No examples, economics or sample structures; aimed at estheticians, not doctors	Doctor-level, clinic-owner-level economics: memberships, LTV, premium positioning for physicians.
Mrinal Tiwari	India; marketing case studies (₹ budgets); 'comment PDF' bait	Unverifiable claims; no strategy shown; India-only	Verified case studies with a baseline, timeframe and attribution method (from your pilot).
Akash Gupta	India; text-led reels; hooks, QUEST, performance marketing, NMC compliance	No worked examples; revenue math not shown; NMC (India) rules only	Worked scripts on screen; UAE (DHA/DoH/MOHAP) and USA (FTC/HIPAA/state boards).
Shivam Bansal	India; research-led doctor visibility and burnout; AI prescription demo	Diagnosis without a playbook; AI demo has no privacy or safety detail	Playbooks + AI demos that show disclosure, consent and human handoff.
Joshua Navaro	Orange County case study: content + Meta ads + automated follow-up	ROI without timeframe or attribution; broad 'DM me' CTA; relies on ads	Organic first, with transparent tracking. Specific CTA ('comment LEAK for a free patient-leak audit').
Dr Sakshi / Jakeyla Reed	Dental systems; aesthetic 'pre-sell with mini case studies'	No templates, screenshots or consult flow shown	Templates + the consult → follow-up → booking flow shown end to end.

The 6 gaps that appear again and again in your data, and that you own

1. **They withhold the value** ('comment X'). → You give the full value in the video and use keywords only for bonuses.
2. **No proof or numbers.** → You show baselines, formulas and pilot results, with honest attribution.
3. **Content and AI are separate worlds.** → You connect them: content → inquiry → AI response → booking → reactivation.
4. **Compliance is India-only or locked away.** → You become the UAE + USA doctor-compliance voice.
5. **Generic 'doctors'.** → You go specialty-specific (derm / cosmetic dental / aesthetics) and city-specific (Dubai, Miami...).
6. **Nobody demonstrates.** → Live audits, mystery-shop tests, script surgery and AI call demos.

3. The Value-Per-Second (VPS) scripting system

The goal: no second of any script is filler. A clinic owner watching for 30 seconds should leave with something they can use or repeat to their team. Here are the rules:

Rule	What it means in practice
1. The 1.5-second triple hook	The visual, written and verbal hooks all fire together and say the same promise. No 'Hi, I'm Dr...', no logo intro, no 'So, today...'.
2. Number or name early	Put a specific number, specialty or city in the first line ('Dubai dermatologists:', '19 hours', '$8,000 veneer case'). Specifics stop the scroll; generic lines don't.
3. One idea per breath	Every sentence must be one of these: a *claim*, a *number*, an *example* or a *step*. If it's none of them, delete it.
4. The delete test	Read the script and remove each sentence in turn. If the video still makes sense without it, cut it permanently. Most doctor scripts lose 30–40% here.
5. Text adds, it doesn't repeat	Captions carry the words. Big on-screen text should carry the *structure* (1/3, 2/3, ❌/✅, the formula) or a number.
6. Open loop by second 5	Promise something the viewer only gets at the end ('...and the third one is the one that costs clinics the most').
7. Show, don't describe	A screen recording of the DM, the bio rewrite or the AI call is worth 10 sentences of explanation.
8. Pattern break every 3–5 seconds	Cut, zoom, B-roll, a text pop or a prop. Change something visual, but only when it supports the point.
9. One CTA, tied to the payload	The CTA continues the value ('Comment LEAK and I'll run this test on your clinic'). Never 'follow for more' on BOFU content.
10. Evidence line	Medical claims get a source in the caption and a doctor-review badge. This is your evidence-based promise, made visible.

The 5-beat script template (30–45 seconds)

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[0–1.5s]  HOOK      Specific promise / pain / number. Visual + text + voice aligned.
[1.5–5s]  STAKES    Why it matters to THEIR clinic (lost patients, money, reputation).
[5–30s]   PAYLOAD   3 steps / 1 demo / before→after. One idea per line. Open loop on step 3.
[30–38s]  PROOF     Example, number, screenshot, or "here's what happened when…"
[38–45s]  NEXT STEP One CTA that extends the value (keyword, save, DM).
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Worked example: Pillar 6, angle 'I messaged 10 Dubai aesthetic clinics at 9 pm'

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HOOK (on screen: "9:00 PM → ? replies")
"I messaged 10 aesthetic clinics in Dubai at 9 pm asking about Botox. Only 3 replied before
morning."

STAKES
"That's a patient who was ready to book, and 7 clinics let her go to sleep thinking about
someone else."

PAYLOAD
"The 3 who replied did one thing right: they answered in under 5 minutes.
 Only one asked me a question back. 'Is this your first time?'
 And only that one offered me two appointment slots. Guess which one I'd have booked." (open
loop closed)

PROOF (screen recording of the 3 replies, names blurred)
"Here's the reply that booked me, and why it works."

NEXT STEP
"Comment LEAK and I'll run this exact test on your clinic, then send you the recording."

Caption: what we tested, how (genuine inquiry, no bookings made), clinic names anonymised.
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> **Ethics note for 'mystery-shop' content**
>
> Keep tests honest. Send a real general question, never book fake appointments, never name or shame a clinic without permission, and blur all names. It protects your reputation with the very clinic owners you want as clients.

PILLAR 1 OF 10 · 15% OF CONTENT · TOFU → MOFU

What Dermatologists, Cosmetic Dentists & Aesthetic Doctors Should Actually Post

Your biggest authority pillar. Clinic owners search for this every week, and it shows you know their specialty from the inside.

Best formats: Talking head + on-screen script, carousels, green-screen over real clinic profiles

Competitor gap: Competitors (Amra Farooq, Akash Gupta, the 'Content ideas for dentists/med spa' reels) list formats like 'myth-busting, BTS, patient stories', but they almost never show a finished, specialty-specific example. Most are generic 'doctors' content, many are India/NMC-focused, and the ideas are hidden behind 'comment CONTENT'.

How you win: Be specialty-specific (derm vs. veneers vs. injectables) and market-specific (Dubai vs. Miami). Show the finished script on screen, and give the idea away in full inside the video. Use the comment keyword only for a bonus, never to unlock the main value.

Value-per-second rule for this pillar

Name the specialty in the first 2 words ('Dermatologists —'). Every idea gets: the idea + one example hook + why it converts. One idea every 4–6 seconds.

1.1 Specialty content blueprints

1	If I ran a Dubai dermatology clinic's Instagram, these are the only 5 videos I'd post this month	Talking head + list	TOFU
2	Cosmetic dentists: 4 videos that sell veneers without showing a single price	Green-screen + examples	MOFU
3	Aesthetic doctors: the 'treatment explained in 30 seconds' series that books consultations	Template breakdown	MOFU

1.2 Treatment-led content (selling high-ticket treatments)

1	How to make a laser / Morpheus8 / HydraFacial video that answers the 3 questions every patient asks before booking	Script on screen	MOFU
2	The 'who is NOT a candidate' video: why telling patients no makes the right ones book	Talking head	TOFU
3	Implants vs. veneers vs. aligners: how to turn patient confusion into a series of 10 videos	Carousel	MOFU

1.3 Content that builds trust before the consultation

1	What happens in your consultation room, shown in 45 seconds, removes the #1 fear of booking	Clinic B-roll + VO	MOFU
2	'What I'd do if this was my own skin/teeth': the most trusted format a doctor can post	Talking head	TOFU
3	Recovery-day reality videos: why showing downtime honestly increases bookings	Day-by-day montage	MOFU

1.4 Seasonal and local calendars (UAE vs. USA)

1	Dubai summer skin content plan: heat, humidity, pigmentation, and the posts your patients search in June	Carousel calendar	TOFU
2	Wedding season, Ramadan and Eid: the content calendar UAE aesthetic clinics keep missing	Talking head	MOFU
3	US clinics: the 'new year, new smile' and pre-summer body content windows, planned 6 weeks early	Calendar walkthrough	MOFU

1.5 What NOT to post

1	5 posts that make a dermatologist look like a skincare influencer and lose patient trust	List + examples	TOFU
2	Stop posting 'Happy Monday' flyers. Here's what to replace each one with	Before/after posts	TOFU
3	Why your 'meet the team' photo gets likes but zero DMs, and the 20-second fix	Teardown	MOFU

PILLAR 2 OF 10 · 10% OF CONTENT · TOFU

Hooks, Scripting & Value-Per-Second (The Craft)

It shows doctors you understand why their videos die in the first 3 seconds. It's also proof of the exact skill they're paying you for.

Best formats: Split-screen script edits, screen recordings, talking head

Competitor gap: Akash Gupta talks about 'QUEST' and hooks without one worked example. 'Comment HOOK for 20 hooks' is withheld value. The scripting reels (how to script as a doctor) start with a framework, then switch to one long melanoma example. Nobody shows a weak script rewritten line by line.

How you win: 'Script surgery': take a real (anonymised) doctor script, cut it live, and show the retention logic. Give hooks matched to specialty and patient intent, not a generic hook bank.

> **Value-per-second rule for this pillar**
>
> Show BEFORE vs AFTER scripts side by side. Count the wasted seconds out loud ('this intro cost you 4 seconds and 60% of viewers').

2.1 Hook engineering

1	Your hook isn't the first sentence. It's the first frame, the first word and the first text line, all at once	Visual breakdown	TOFU
2	I rewrote a dermatologist's hook 5 ways. Here's which one patients would actually stop for	Split screen	TOFU
3	Doctors: stop starting videos with 'Hi, I'm Dr...'. Here's what to say in your first 1.5 seconds	Talking head	TOFU

2.2 Script surgery (live rewrites)

1	This 60-second veneer script had 22 seconds of filler. Watch me cut it	Screen recording	MOFU
2	Turning a boring 'benefits of retinol' video into a story patients finish	Before/after script	TOFU
3	The 4-line script structure I use for every doctor: hook, stakes, payload, next step	Template on screen	MOFU

2.3 Retention and pacing

1	Why patients scroll at second 7, and the 'open loop' that keeps them watching	Talking head + graphic	TOFU
2	On-screen text should never repeat what you're saying. Here's what it should do instead	Demo	TOFU
3	The 'one idea per breath' rule for doctors who over-explain	Talking head	TOFU

2.4 Outlier research (templatising winners)

1	How I find viral dermatology videos to model: the 5x outlier rule explained	Screen recording	MOFU
2	Copying a viral format without copying the content: the 70/20/10 rule for doctors	Talking head	MOFU
3	One outlier video → 10 scripts for a cosmetic dentist, built in front of you	Screen recording	MOFU

2.5 Camera confidence for doctors

1	Doctors who hate the camera: film in 'consultation mode' instead of 'presenter mode'	Talking head	TOFU
2	How a busy dermatologist films 40 videos in two 3-hour sessions a month	BTS of filming day	MOFU
3	You don't need to be charismatic on camera. You need to be clear. Here's the difference	Talking head	TOFU

PILLAR 3 OF 10 · 15% OF CONTENT · MOFU

Views → Patients (Content as a Patient-Acquisition Channel)

This sets you apart from every 'grow your Instagram' creator. Doctors don't want followers. They want booked consultations.

Best formats: Whiteboard/funnel graphics, Instagram Insights screen recordings, talking head

Competitor gap: Amra Farooq's 'relevance over reach' and 'patients may already be watching' have no clinic example, no conversion numbers and no audit. Joshua Navaro shows ROI but no attribution method. Most 'views-to-bookings' med-spa reels never show the booking step.

How you win: Show the full path, with numbers: views → profile visits → DMs → consultations → treatments. Teach a tracking method (keyword DMs, 'how did you hear' at intake, call-tracking). Be honest that you set a baseline before you claim results.

> **Value-per-second rule for this pillar**
>
> Each video gives one metric, one formula, or one fix the doctor can apply today. Put the number on screen in the first 2 seconds.

3.1 Views vs. patients

1	A 200k-view Reel with zero consultations is a failed Reel. Here's how to tell before you post	Talking head	TOFU
2	The 3 numbers I check before I check views: profile visits, DMs, link taps	Insights screen rec	MOFU
3	Why your viral skincare-myth video brought followers from 40 countries and no patients from Dubai	Talking head	TOFU

3.2 Profile conversion

1	Your bio fails the 5-second test. Line-by-line rewrite for a cosmetic dentist	Profile teardown	MOFU
2	Highlights are your sales page. The 5 highlights every aesthetic clinic needs	Screen recording	MOFU
3	Pinned posts: the 3 videos a new patient should see first on your profile	Talking head	MOFU

3.3 Calls-to-action that patients use

1	'Link in bio' is dead for clinics. Use this DM-keyword CTA instead	Demo	MOFU
2	Soft CTA vs. hard CTA: when a dermatologist should ask for the booking	Talking head	MOFU
3	The comment-keyword → instant reply → booking flow, shown end to end	Screen recording	BOFU

3.4 Tracking and attribution

1	How to know which Reel brought which patient (without expensive software)	Talking head + sheet	MOFU
2	The one question your front desk must ask every new patient	Talking head	MOFU
3	My monthly clinic growth report: what I show doctors instead of 'we posted 40 Reels'	Report walkthrough	BOFU

3.5 Content → inquiry economics

1	If one veneer case is worth $8,000, how many views do you actually need? Let's do the math	Whiteboard	MOFU
2	Small accounts book more patients than big ones. Here's why local relevance wins	Talking head	TOFU
3	1 topic → 6 videos → 1 consultation: how a content cluster sells a high-ticket treatment	Diagram	MOFU

PILLAR 4 OF 10 · 10% OF CONTENT · TOFU → MOFU

Patient Psychology: How Patients Choose a Dermatologist or Cosmetic Dentist

As a doctor, you can explain patient fear, trust and decision-making with authority no marketer has. This is your clinical edge in content form.

Best formats: POV skits, talking head, patient-journey diagrams

Competitor gap: Shivam Bansal ('the brain chooses who feels safe') and Jakeyla Reed ('sell before the consultation') touch on this but give no specialty examples or consult flow. Maha Chalawi's luxury/high-paying-client reels have no positioning examples or pricing language.

How you win: Map the real patient decision journey for high-ticket aesthetic and dental treatments (fear of looking 'done', pain, cost, regret). Then show the content that answers each fear. Draw on what you learned in medical school about communication.

Value-per-second rule for this pillar

Open with the patient's exact inner thought in quotes ('Will it look fake?'), then give the content answer.

4.1 The fears behind high-ticket treatments

1	'Will it look fake?' The fear stopping your filler patients, and the video that removes it	POV + talking head	TOFU
2	Dental anxiety is why veneer inquiries stall. 3 videos that calm it before the visit	Talking head	MOFU
3	Price isn't the objection. Regret is. How to address regret in your content	Talking head	MOFU

4.2 Trust signals patients look for

1	Patients decide in 7 seconds on your profile. These are the trust signals they scan for	Profile walkthrough	TOFU
2	Why patients trust your face more than your clinic logo	Talking head	TOFU
3	Credentials vs. clarity: which builds more trust on Instagram?	Talking head	TOFU

4.3 Premium positioning without discounts

1	Stop attracting price shoppers. Content that attracts patients who value expertise	Talking head	MOFU
2	Why discount flyers quietly damage a dermatologist's reputation	Talking head	TOFU
3	How luxury clinics in Dubai talk about price, and what US clinics can learn	Comparison	MOFU

4.4 Decision journey and timing

1	Most patients watch you for weeks before they DM. Here's what they need to see in that time	Diagram	MOFU
2	The 'research phase' patient: content for people comparing 3 clinics	Talking head	MOFU
3	Why the patient who asks 'how much?' isn't cold, and how to answer without losing them	Script demo	MOFU

4.5 Doctor–patient communication (your medical edge)

1	What medical school taught me about patient trust that marketers don't know	Talking head	TOFU
2	Words that scare patients in clinic videos, and what to say instead	List	TOFU
3	Explaining risks honestly on Instagram makes you more bookable, not less	Talking head	MOFU

PILLAR 5 OF 10 · 5% OF CONTENT · MOFU

Evidence-Based & Compliant Medical Marketing (UAE + USA)

This is your strongest defensible gap. Generic agencies can't do it, and doctors are most afraid of getting it wrong. 'Evidence-based, doctor-reviewed content' is your core promise.

Best formats: Split-screen rewrites, carousels, talking head

Competitor gap: Every compliance reel in your data is India/NMC-focused (Akash Gupta, Shivam Bansal) or a withheld checklist (Amra Farooq 'comment Checklist'). Nobody in your dataset covers DHA/DoH/MOHAP rules in the UAE or FTC/HIPAA/state-board rules in the USA. Your notes repeatedly flag 'add compliance disclaimer' as a white space.

How you win: Be 'the doctor who knows the rules in Dubai and Miami'. Show compliant vs. non-compliant versions of the same post. Always add 'educational, not legal advice' and verify each rule against the regulator's current guidance before filming.

Value-per-second rule for this pillar

Split screen: ❌ risky claim / ✅ compliant rewrite. One rule per video, with the regulator named on screen.

5.1 UAE medical advertising rules

1	Running a clinic in Dubai? Your Instagram ads may need approval first. What clinic owners should check with DHA	Talking head + carousel	MOFU
2	Dubai vs. Abu Dhabi vs. other emirates: who regulates your clinic's marketing (DHA, DoH, MOHAP)	Carousel	MOFU
3	Doctors posting paid collaborations in the UAE: what to know about advertiser permits	Talking head	MOFU

5.2 USA rules (FTC, HIPAA, state boards)

1	That patient selfie you reposted might be a HIPAA problem. What written authorization should cover	Talking head	MOFU
2	'Cosmetic dentistry specialist': why that phrase can get US dentists in trouble with their state board	Talking head	MOFU
3	Fake or incentivised reviews: what the FTC's rules mean for clinics asking for Google reviews	Carousel	MOFU

5.3 Before/after and testimonial ethics

1	Before/afters that build trust vs. ones that break rules: same lighting, same angle, real consent	Split screen	MOFU
2	How to film a patient testimonial with consent that holds up in both the US and UAE	Checklist walkthrough	MOFU
3	'Results may vary' isn't enough. How to present outcomes honestly	Talking head	TOFU

5.4 Evidence-based content

1	I fact-check every script we make. Here's what that process looks like	BTS screen recording	BOFU
2	Viral skincare claims dermatologists should stop repeating, and what the evidence says	Myth-bust	TOFU
3	Why ChatGPT-written medical scripts are risky, and how doctors should use AI safely	Talking head	TOFU

5.5 Claims and wording

1	'Painless', 'permanent', 'guaranteed': words to remove from your clinic content today	List	TOFU
2	Rewriting 5 risky aesthetic claims into compliant ones that still sell	Split screen	MOFU
3	Educational vs. promotional content: why the line matters for doctors	Talking head	MOFU

PILLAR 6 OF 10 · 10% OF CONTENT · MOFU → BOFU

The Patient Leak: Inquiry → Booking (Speed, DMs, Front Desk)

This is the bridge between your two offers. 'Content brings the opportunity; systems decide what happens to it.' It's the most direct path to a sales call.

Best formats: Screen recordings of inquiries, call recordings (with consent), talking head

Competitor gap: Sam R Patel owns 'front desk' content (greetings, call reviews), but rarely names fixes, scripts or metrics, and has no AI angle. Dr. Nickc teases a follow-up system ('comment FOLLOWUP') without showing it. Nobody does live 'mystery-shop' tests on real clinic DMs and calls.

How you win: Run 'Patient Leak Tests': send a genuine inquiry to a clinic's DM, WhatsApp, form or phone (anonymised, no fake bookings), time the response, and show the fix. Then show how an AI layer closes the gap.

> **Value-per-second rule for this pillar**
>
> Put a timer on screen: 'Inquiry sent 8:42 pm. Reply received: 19 hours later.' The number is the hook.

6.1 Speed to lead

1	I messaged 10 Dubai aesthetic clinics at 9 pm. Here's how many replied before morning	Experiment series	TOFU
2	Every hour your clinic waits to reply, a patient books somewhere else. Here's why	Talking head	MOFU
3	After-hours inquiries are your highest-intent patients. Here's who answers them right now	Talking head	MOFU

6.2 DM and WhatsApp conversion

1	The 'How much is Botox?' DM: 3 replies that lose the patient, 1 that books them	Screen rec + script	MOFU
2	WhatsApp is the UAE's front desk. How clinics should structure replies	Demo	MOFU
3	Don't send a price list. Send this 3-message sequence instead	Script on screen	MOFU

6.3 Phone and front-desk scripts

1	Stop asking 'When would you like to come in?' Offer two slots instead	Role-play	MOFU
2	Rating a real (anonymised) clinic call: 4 moments where the booking was lost	Call review	MOFU
3	Why your receptionist isn't the problem, the system around them is	Talking head	MOFU

6.4 Consult → treatment conversion

1	Patients who don't book after a consultation aren't 'no'. They're 'not yet'. The follow-up plan	Talking head	MOFU
2	The 3-day, 7-day, 21-day follow-up that recovers 'I'll think about it' patients	Timeline graphic	BOFU
3	Treatment-plan recovery: the patients already sitting in your 'pending' list	Talking head	BOFU

6.5 Leak audits

1	Where your clinic loses patients: the 7-point leak map from content to returning patient	Diagram	MOFU
2	I audited a cosmetic dentist's patient journey. Here are the 3 biggest leaks	Teardown	BOFU
3	Self-audit: 5 questions to find out if your clinic has a follow-up problem	Carousel	MOFU

PILLAR 7 OF 10 · 15% OF CONTENT · MOFU → BOFU

AI Systems for Clinics (Receptionist, Booking, No-Shows)

Your second authority pillar. Always connect AI to a clinic outcome, never 'cool automation'.

Best formats: Screen recordings, voice-agent call demos, before/after workflow diagrams

Competitor gap: AI reels in your dataset are generic ('AI agent replaces receptionist', '$120M raise for voice agents') or product promos with no medical privacy, escalation or handoff detail. Shivam Bansal's AI prescription demo has no safety or consent detail. Dr. Nickc says 'build systems around AI' but shows none.

How you win: Show working demos built for clinics, including the safety layer: disclosure that it's AI, human handoff, no clinical advice, consent for messages, and data handling. A doctor building AI with clinical guardrails is a rare position.

Value-per-second rule for this pillar

Demo first, explanation second: open with the AI answering a real-style patient question in 3 seconds, then explain.

7.1 AI receptionist and missed calls

1	Watch an AI receptionist answer a Dubai clinic's call at 11 pm, in Arabic and English	Live demo	BOFU
2	Missed-call text-back: the simplest AI system every clinic should have first	Demo	MOFU
3	What an AI receptionist should never say to a patient (my doctor's guardrails)	Talking head	MOFU

7.2 Lead qualification and booking

1	An AI that asks the 4 questions your receptionist forgets, then books the slot	Demo	BOFU
2	How AI qualifies a veneer inquiry without sounding robotic	Chat demo	MOFU
3	Instant DM reply → qualification → calendar booking: the full flow in 40 seconds	Screen recording	BOFU

7.3 No-shows and reminders

1	Your reminder text is why patients don't show up. The 48h / 24h / day-of sequence that works	Script on screen	MOFU
2	Turning cancellations into filled slots with an automated waitlist	Demo	BOFU
3	Deposits, confirmations and reminders: reducing no-shows without annoying patients	Talking head	MOFU

7.4 Safe AI in healthcare

1	AI should make your receptionist better, not replace them. Here's what that looks like	Talking head	TOFU
2	Patient data and AI tools: questions every US clinic should ask a vendor about HIPAA	Carousel	MOFU
3	Automated texts and AI calls in the US need consent. What clinic owners should know	Talking head	MOFU

7.5 What to automate (and what not to)

1	Don't automate everything. The 6 clinic tasks AI should handle, and the 3 it shouldn't	List	TOFU
2	An AI receptionist won't fix a broken patient journey. Fix this first	Talking head	MOFU
3	The order I'd install AI in a new clinic: week 1, week 2, week 4	Roadmap graphic	BOFU

PILLAR 8 OF 10 · 5% OF CONTENT · MOFU → BOFU

Retention, Reactivation & Reviews (The Money Already in the Database)

For established clinics, past patients are the fastest ROI and the easiest pilot win. It makes your AI offer feel concrete.

Best formats: Message-sequence screenshots, talking head, CRM screen recordings

Competitor gap: Reactivation reels ('reactivate patients who haven't booked in 2 years', 'best & worst ways') give scripts but no segmentation, consent, cadence or results. Maha Chalawi's loyalty reels have no economics or sample structure. Review reels skip compliance (incentives, review gating).

How you win: Show reactivation by treatment cycle (Botox ~3–4 months, HydraFacial monthly, hygiene 6 months, retainer checks) with timing logic you know clinically. Then show the message, the channel, the consent and the result.

Value-per-second rule for this pillar

Lead with the treatment cycle as a number: 'Your Botox patients are due back in about 3–4 months. Most clinics never message them.'

8.1 Database reactivation

1	You have patients in your system who haven't visited in 12 months. Here's the first message I'd send	Script on screen	BOFU
2	Reactivation by treatment cycle: why one message for everyone fails	Talking head	MOFU
3	Before you run a 'we miss you' campaign, segment your list like this	CRM demo	MOFU

8.2 Rebooking and recall

1	Recall timing that respects the treatment, explained by a doctor	Talking head	MOFU
2	Post-treatment follow-up on day 1, day 7 and day 30, and why each one matters clinically	Timeline	MOFU
3	The rebooking question your clinic should ask before the patient leaves	Role-play	MOFU

8.3 Reviews and reputation

1	How to ask for Google reviews without breaking the rules	Script	MOFU
2	Your first 50 reviews: the automated request flow, timed after the result shows	Demo	BOFU
3	How to reply to a 1-star review as a doctor (without breaching confidentiality)	Talking head	MOFU

8.4 Loyalty, memberships and referrals

1	Loyalty programs that build value, not discount addiction	Talking head	MOFU
2	Skin memberships: how dermatology clinics turn one-off visits into recurring revenue	Breakdown	MOFU
3	A referral ask that feels natural, timed at the moment patients love their result	Script	MOFU

8.5 Lifetime value

1	Why a $300 facial patient can be worth thousands over 3 years, and how clinics lose them	Whiteboard	MOFU
2	Retention beats acquisition. The math for aesthetic clinics	Whiteboard	MOFU
3	The week-one experience decides if a patient returns. What to automate in those 7 days	Talking head	MOFU

PILLAR 9 OF 10 · 5% OF CONTENT · MOFU → BOFU

Clinic Growth Economics, Teardowns & Proof

This makes you a strategic partner, not a vendor. Teardowns and case studies are your highest-converting BOFU content.

Best formats: Whiteboard, profile teardowns, case-study walkthroughs, dashboards

Competitor gap: Mrinal Tiwari's case studies ('rejected a 30 lac project', 'Mumbai derm budget 52 lac') are unverifiable and gated behind 'comment PDF'. Joshua Navaro's ROI claims have no timeframe or attribution. Dr. Nickc's KPI reels tease 'if you know these...' without naming them. Nobody shows the formulas.

How you win: Name the KPIs and show the formulas and assumptions openly. Do public profile teardowns of real (permission-based or anonymised) clinics. Publish pilot results with baseline, timeframe and attribution method, which is exactly what competitors leave out.

> **Value-per-second rule for this pillar**
>
> Write the formula on screen. Example: 'Patient value × booked consults × show rate = this month's content ROI.'

9.1 Clinic KPIs

1	The 6 numbers every aesthetic clinic owner should know every Monday	Carousel + talking head	MOFU
2	Inquiry-to-booking rate: the KPI that tells you if marketing or follow-up is broken	Whiteboard	MOFU
3	Cost per new patient: how to calculate it honestly (organic + paid)	Whiteboard	MOFU

9.2 Profile and clinic teardowns

1	Teardown: a Miami dermatologist with 50k followers and almost no booking path	Green-screen teardown	MOFU
2	Teardown: why this Dubai dental clinic's content looks expensive but converts poorly	Green-screen teardown	MOFU
3	Rating 3 cosmetic dentist profiles out of 10, and what I'd change first	Ranking	TOFU

9.3 ROI math

1	Is a $2,000/month content + AI system worth it? Let's calculate for an implant clinic	Whiteboard	BOFU
2	Why more leads isn't always the answer. The math of fixing conversion first	Whiteboard	MOFU
3	Referrals vs. content vs. ads: what each patient source really costs	Comparison	MOFU

9.4 Case studies (from your pilot)

1	60 days with a [specialty] clinic: baseline, what we changed, what happened	Case study	BOFU
2	The video that brought one clinic its most valuable consultation, and why it worked	Breakdown	BOFU
3	What didn't work in our first pilot, and what we changed	Honest recap	BOFU

9.5 Owner strategy

1	Scaling a private practice: hire more staff or build systems first?	Talking head	MOFU
2	If your clinic depends 100% on referrals, your growth is left to chance	Talking head	TOFU
3	Launching a new treatment? The 30-day content + follow-up launch plan	Roadmap	BOFU

PILLAR 10 OF 10 · 10% OF CONTENT · TOFU → MOFU

Dr. Azka: A Doctor Building for Doctors (Human Layer)

Your personal brand. It turns 'an agency' into 'the doctor I trust'. Together with Pillar 1 it makes up the 20% human side of your 80/20.

Best formats: Vlogs, voiceover B-roll, talking head, 'come build with me' series

Competitor gap: Amra Farooq's origin story ('This is my story :)') has no turning point or proof. Shivam Bansal and the 'doctor-turned-entrepreneur' reels are inspirational but vague. Very few creators are young female doctors building for UAE + US clinics. Your story (doctor at 24, failed e-commerce/agency, found AI + content) is distinctive.

How you win: Build in public: real numbers, real failures, real client lessons. Tie every lifestyle clip back to your identity ('a doctor building a business for doctors').

Value-per-second rule for this pillar

Even story videos need a payoff line. Every personal video ends with one lesson a clinic owner can use.

10.1 Origin story

1	I'm a doctor at 24 and I chose not to practise clinically, yet. Here's why	Talking head	TOFU
2	In medical school I realised doctors need to be online to stand out. Nobody taught us how	Story + B-roll	TOFU
3	I didn't leave healthcare. I found another way to contribute to it	Talking head	TOFU

10.2 Failures and lessons

1	My first agency failed. Here's what I got wrong	Story	TOFU
2	I tried e-commerce before this. What it taught me about patients and customers	Story	TOFU
3	Why I stopped building a business for everyone and chose only clinics	Talking head	MOFU

10.3 Building in public

1	Day 1 of taking my agency from $0 to $10k/month, working only with clinics	Series	TOFU
2	Week 4 update: clients, calls, mistakes and numbers	Series	MOFU
3	What happened when I cold-messaged 100 dermatologists	Story + data	MOFU

10.4 Day in the life

1	A day in my life as a doctor running a content + AI agency in Dubai	Vlog	TOFU
2	Behind the scenes of scripting 40 videos for one doctor	BTS	MOFU
3	A filming day at a client clinic, from our side	BTS vlog	MOFU

10.5 Opinions and beliefs

1	Unpopular opinion: doctors don't need more followers. They need more patients	Talking head	TOFU
2	Most clinics don't have a lead problem. They have a follow-up problem	Talking head	MOFU
3	Content gets you the opportunity. Your systems decide what happens to it	Signature talk	MOFU

4. Your first 30 days (40 pieces): a suggested sequence

This follows your brand document's goal of '30 days → 40 pieces', then review. Weeks 1–2 build discovery and trust; weeks 3–4 add proof and conversion as your pilot starts producing results.

Week	Focus	Mix (≈10 pieces/week)
1	Who you are + your POV	3× Pillar 10 (origin, why clinics, signature line) · 3× Pillar 1 · 2× Pillar 3 · 2× Pillar 2
2	The leak (your bridge)	3× Pillar 6 (start the 'I messaged 10 clinics' series) · 2× Pillar 4 · 2× Pillar 5 · 2× Pillar 1 · 1× Pillar 10
3	AI with guardrails	3× Pillar 7 (live demos) · 2× Pillar 8 · 2× Pillar 3 · 2× Pillar 2 · 1× Pillar 10 (build-in-public update)
4	Proof & teardowns	3× Pillar 9 (teardowns, ROI math, first pilot notes) · 2× Pillar 6 · 2× Pillar 7 · 2× Pillar 1 · 1× Pillar 10

Repurposing rule (1 → 4)

- **Instagram Reel** (main) → **carousel** of the same payload → **LinkedIn text post** (clinic owners and practice managers are there) → **email** to your list or prospects. Your outreach in Document 2 reuses these same pieces as 'value-first' touchpoints.

Monthly review (templatise your winners)

- Track for each post: views, profile visits, follows, saves/shares, **DMs/keyword comments**, and **calls booked**.
- Any post that does 3–5× your average becomes a **template**. Keep the hook structure and format, and change the topic or specialty (the 70/20/10 rule: 70% proven outlier structures, 20% your own winners, 10% experiments).

> **Before filming any compliance or regulation angle**
>
> The regulatory angles in Pillar 5 (and the consent angles in Pillar 7) name real bodies and rules: DHA, DoH, MOHAP, UAE advertiser permits, FTC, HIPAA, state boards, US consent rules for automated texts and calls. Rules change. Check each one against the regulator's current official guidance and add 'educational, not legal advice' to the caption.